

Policy on Monitoring after Medication Administration

«Hospital Name»

Document control

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Prepared by	Medication Safety Officer Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

A blank marked _____ must be completed before issue.

Statement of intent

Every patient is monitored after medication administration to confirm the intended effect, high-risk patients are monitored regularly, and near misses, medication errors, and adverse drug reactions are captured, reported, analysed, and acted on.

1. Purpose

This policy explains how «Hospital Name» monitors patients after medication administration, adjusts medication based on that monitoring, and captures, reports, analyses, and acts on near misses, medication errors, and adverse drug reactions.

This policy does not cover medication administration or prescription in detail — those are covered in other hospital policies.

2. Scope

This policy applies to all clinical staff involved in monitoring patients after medication administration at «Hospital Name».

3. Policy standards

«Hospital Name» monitors patients collaboratively after medication administration to confirm the intended effect, defines which situations and medications need closer monitoring, monitors high-risk patients — including those on dialysis, in the ICU, and the

elderly — regularly, and changes medication based on monitoring where appropriate. Near misses, medication errors, and adverse drug reactions are defined, captured under written guidance, reported within a defined time frame, analysed by the multidisciplinary committee, and followed up with corrective or preventive action where appropriate.

Staff follow the written guidance below and keep the records it requires.

4. Non-negotiable rules

1. Post-administration monitoring is required for every patient following medication administration; the organisation must maintain written guidance defining the parameters to monitor, the observation intervals, and the criteria requiring escalation to a clinician.
2. Do not leave high-risk patients — those on dialysis, in the ICU, or the elderly — without regular monitoring of medication effects.
3. Do not operate without written guidance defining near misses, medication errors, and adverse drug reactions, and without a defined process for identifying, documenting, reporting, analysing, and acting on them.
4. Do not leave the reporting time frame for near misses, medication errors, or adverse drug reactions undefined, and do not miss the defined time frame once it's set.
5. Collected near-miss reports, medication error reports, and adverse drug reaction reports must be subjected to multidisciplinary committee analysis; no collected event report may remain unreviewed, and findings that require corrective or preventive action must be resolved within the organisation's defined timeframe.

5. What we do

5.1 Monitor patients after medication administration

Monitoring after administration is done collaboratively, to confirm the medicine is having its intended effect — this could include monitoring through laboratory results. Monitoring also helps identify near misses, medication errors, and adverse drug reactions. «Hospital Name» defines the situations and medications that call for more frequent monitoring — for example, administration of high-risk medicines. Medication effects in high-risk patients — those on dialysis, in the ICU, and the elderly — are monitored on a regular basis; these are examples of high-risk groups, not an exhaustive list.

5.2 Change medication based on monitoring

Where appropriate, medications are changed based on clinical response and any adverse drug reactions identified through monitoring.

5.3 Capture near misses, medication errors, and adverse drug reactions

Near misses, medication errors, and adverse drug reactions are defined by «Hospital Name», in line with best practice. Written guidance sets out the process for capturing them, covering five steps: identifying, documenting, reporting, analysing, and taking action.

5.4 Report within a defined time frame

«Hospital Name» defines the time frame for reporting a near miss, medication error, or adverse drug reaction once it occurs, and adheres to that time frame.

5.5 Collect and analyse

Details of near misses, medication errors, and adverse drug reactions are collected and analysed by the multidisciplinary committee, within a defined time frame. It's preferable for a clinical pharmacologist or clinical pharmacist to take part in this analysis.

5.6 Take corrective and preventive action

Where appropriate, corrective or preventive action is taken based on the analysis, and records of these actions are kept. It's preferable for corrective and preventive action to be based on root-cause analysis.

6. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable for ensuring post-administration monitoring and the near-miss/error/ADR capture system are resourced and implemented.
Medication Safety Officer	Owns day-to-day implementation of this policy; coordinates the capture, reporting, and analysis of near misses, medication errors, and ADRs; brings findings to the multidisciplinary committee.
Multidisciplinary committee (DTC)	Analyses near misses, medication errors, and ADRs within the defined time frame; takes or approves corrective and preventive action.
Treating doctors / clinical staff	Monitor patients collaboratively after medication administration; change medications based on monitoring where appropriate; report near misses, medication errors, and ADRs within the defined time frame.
Nursing staff	Monitor patients after administration; capture and report near misses, medication errors, and ADRs per the defined process and time frame.
Quality Coordinator	Audits this policy; holds training records

	and staff acknowledgements.
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7. Quality monitoring

Monitoring area	What is monitored
Post-administration monitoring	Done collaboratively; situations and medications needing more frequent monitoring defined; high-risk patients (those on dialysis, in the ICU, and the elderly) monitored regularly; monitoring records present.
Medication changes from monitoring	Changes based on clinical response and ADRs where appropriate; rewritten orders present.
Near-miss/error/ADR definitions and capture guidance	Definitions in place and in line with best practice; written guidance covering all five steps (identifying, documenting, reporting, analysing, acting on); MOM.8.c is CORE — absence of definitions or a working capture system is accreditation-blocking; no stop-work section applies.
Reporting time frame	Time frame defined by the organisation; adherence evidenced by comparing event date to report date on a sample of incident forms.
Multidisciplinary committee analysis	Analysis conducted within the defined time frame; DTC record shows collection and analysis, not just individual review.
Corrective and preventive action	CAPA records maintained where appropriate; open actions tracked.

8. Training and staff acknowledgement

All clinical staff involved in monitoring patients after medication administration shall be familiar with the monitoring requirements, the near-miss/error/ADR definitions, the capture and reporting process, the defined reporting time frame, and the roles in analysis and action in this policy.

I have read the Policy on Monitoring after Medication Administration of «Hospital Name». I will follow the processes described.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

9. Distribution

This policy shall be available to all clinical staff who monitor patients after medication administration, the Medication Safety Officer, the multidisciplinary committee, and the Quality Coordinator.

10. Abbreviations

Abbreviation	Meaning
ADR	Adverse drug reaction
CAPA	Corrective and Preventive Action
CORE	Core objective element (NABH)
DTC	Drug and Therapeutics Committee (the organisation's multidisciplinary medication management committee)
ICU	Intensive Care Unit
MOM	Management of Medication (NABH Hospitals chapter)
MSO	Medication Safety Officer
NABH	National Accreditation Board for Hospitals and Healthcare Providers

11. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

Objective Element	Level	Traceability to this policy
MOM.8.a	Commitment*	Sections 3 and 5.1 address collaborative post-administration monitoring, the organisation's definition of situations and medications requiring more frequent monitoring, and the three named high-risk patient categories: those on dialysis, in the ICU, and the elderly.
MOM.8.b	Commitment	Section 5.2 addresses medication changes based on clinical response and adverse drug reactions identified through monitoring, where appropriate.
MOM.8.c	CORE*	Sections 3 and 5.3 address the definition of near

		misses, medication errors, and adverse drug reactions in line with best practice, and written guidance covering all five mandatory capture steps: identifying, documenting, reporting, analysing, and acting on them. MOM.8.c carries CORE status — this policy does not contain a stop-work section, which is correct, as MOM.8 is not in the MOM stop-work proposals.
MOM.8.d	Commitment*	Sections 3 and 5.4 address the definition of the reporting time frame and the requirement to adhere to it. The specific time frame is set by «Hospital Name» — this policy does not substitute a specific number of hours or days.
MOM.8.e	Commitment	Section 5.5 addresses collection and analysis by the multidisciplinary committee within a defined time frame, with a preference for clinical pharmacologist or pharmacist involvement.
MOM.8.f	Commitment	Section 5.6 addresses corrective or preventive action taken where appropriate, with records kept, and a preference for root-cause analysis as the basis.

12. Required Records/Evidence Checklist

Post-administration monitoring — MOM.8.a (Commitment*)

1. Written monitoring guidance naming what to watch for and when to escalate, matched to the drug and clinical setting.
2. Record of situations and medications where more frequent monitoring is required, as defined by the organisation.

3. Recorded monitoring for a sample of patients — including high-risk patients (those on dialysis, in the ICU, and the elderly) — after medication administration.

Medication changes from monitoring — MOM.8.b

1. Medication-change records based on monitoring findings — drug changed, held, or dose adjusted based on clinical response or ADR.
2. Rewritten-order records for any medication change.
3. Confirmation the treating doctor was informed and the situation was not silently continued.

Near-miss/error/ADR capture — MOM.8.c (CORE*)

1. Written definitions of near misses, medication errors, and adverse drug reactions, in line with best practice.
2. Written guidance setting out the five mandatory capture steps: identifying, documenting, reporting, analysing, and acting on.
3. Sample of captured near-miss and incident reports demonstrating the process is working.

Reporting time frame — MOM.8.d (Commitment*)

1. Defined reporting time frame for near misses, medication errors, and adverse drug reactions — as set by the organisation.
2. Adherence evidence: comparison of event date/time to report date/time on a sample of incident forms.

Multidisciplinary committee analysis — MOM.8.e

1. DTC analysis records showing details of near misses, medication errors, and ADRs collected and analysed as a committee.
2. Defined analysis completion time frame, evidenced in committee minutes or terms of reference.

Corrective and preventive action — MOM.8.f

1. CAPA records where corrective or preventive action was taken — with owner and due date.
2. Open-action tracking records.
3. Decision record where CAPA was considered but judged not required, where that is the case.

13. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. MOM.8.

Disclaimer

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The requirements in this document are accreditation requirements of the NABH Full Accreditation Standards for Hospitals, 6th Edition, not duties under a named Act of Parliament. This policy does not import the Consumer Protection Act, 2019, the Clinical Establishments Act, 2010, or the Mental Healthcare Act, 2017 as a checklist. Statutory duties that arise under other documents of «Hospital Name» remain those documents. «Hospital Name» is responsible for verifying any statutory duty that applies to it; this document does not constitute legal advice.

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